

**Symptoms and Treatment of Overdose:**

Overdosage of Amlodipine might lead to excessive peripheral vasodilation with marked hypotension.

Hypotension due to severe overdosage should be treated with active cardiovascular support by frequently monitoring the blood pressure, elevation of extremities, and judicious administration of fluid. Next, administration of a vasoconstrictor may be useful provided there is no contraindication. Intravenous calcium gluconate may be used to reverse the calcium channel blockade effect.

As amlodipine is highly protein-bound, hemodialysis is not likely to be of benefit.

**Storage Condition:**

Store in a dry place (below 30°C). Protect from light.

**Pack Size:**

Blister pack of 3 x 10's, 10 x 10's and 50 x 10's.

**Product Registration Number:**

**Amlo-5 Tablet:** MAL07011015AZ

**Amlo-10 Tablet:** MAL07011016AZ

Further information can be obtained from your doctor or pharmacist.

**Product Holder / Manufactured by:**

**ROYCE PHARMA MFG. SDN.BHD. (650435-X)**

PT 1663, Nilai Industrial Estate,

71800 Nilai, Negeri Sembilan, Malaysia.

Revision date: 220318

ROYCE®

## AMLO TABLET

(Amlodipine Besylate-Antihypertensive)

**Presentation:**

White to off white oblong shaped tablet with score line on one side.

**Amlo-5 Tablet:** Each tablet contains Amlodipine Besylate equivalent to Amlodipine 5mg.

**Amlo-10 Tablet:** Each tablet contains Amlodipine Besylate equivalent to Amlodipine 10mg.

**Pharmacological Information:**

Amlodipine as a calcium channel blocker is indicated for treatment of hypertension and in controlling the blood pressure in patients as well as the prophylaxis of angina.

Amlodipine relaxes vascular smooth muscle and dilates coronary and peripheral arteries. It has more influence on vessels and less on the myocardium and has no anti-arrhythmic activity. It rarely precipitates heart failure because any negative inotropic effect is offset by a reduction in left ventricular work. Amlodipine does not reduce myocardial contractility and it does not produce clinical deterioration in heart failure. Unlike the other antihypertensive drugs, Amlodipine has a longer duration of action. In patients with hypertension, once daily dosing provides clinically significant reductions of blood pressure in both the supine and standing positions throughout the 24 hour interval. In patients with angina, once daily administration of Amlodipine increases total exercise time, time to angina onset, and time to 1mm ST segment depression, and decreases both angina attack frequency and nitroglycerine tablet consumption.

After oral administration of therapeutic doses of Amlodipine, absorption produces peak plasma concentrations between 6 and 12 hours. Absolute bioavailability has been estimated to be between 64 and 80%. The bioavailability of Amlodipine is not altered by the presence of food.

Amlodipine is extensively (around 90%) converted to inactive metabolites via hepatic metabolism with 10% of the parent compound and 60% of the metabolites excreted in the urine. Oxidation of the pyridine analogue, with subsequent oxidative deamination of the 2-aminoethoxymethyl side chain or de-esterification at the 5-methoxycarbonyl group; rate of oxidation of Amlodipine may be slower, as suggested by less extensive and variable first-pass metabolism and a longer half-life compared to other dihydropyridines.

**Indication:**

Amlo is indicated for the treatment of hypertension, chronic stable angina and vasospastic angina (Prinzmetal's or variant angina) either as a monotherapy or in combination with other antihypertensive or antianginal drugs.

**Dosage and Administration:**

In hypertension or angina, an initial dosage of 5mg once daily which may be increased to a maximum dosage of 10 mg daily depending on the patient's response.

In children, the safety and effectiveness of Amlodipine have not been established.

Dosage adjustment is not required in elderly and in patients with renal failure.

In patients with Impaired Hepatic Function, the half-life of Amlodipine is prolonged. However, the recommended dose has not been established. Therefore, the drug should be administered with caution in these patients.

Dose adjustment is not essential for concomitant treatment of Amlodipine with thiazide diuretics, beta-blockers, as well as angiotension-converting enzyme (ACE) inhibitors.

**Route of Administration:**

For oral use only.

**Contraindication:**

Amlodipine is contraindicated in patients with known hypersensitivity to amlodipine, cardiogenic shock, unstable angina and significant aortic stenosis.

**Warning and Precautions:**

Amlodipine should be used with care in patients with impaired liver function as amlodipine has a prolonged half life in these patients.

**Pregnancy and Lactation:**

The safety of Amlodipine in pregnancy and nursing mother has not been established. Animal teratology and reproduction studies have demonstrated no adverse effects other than to delay parturition and prolong labor in rats at a dose level fifty times the maximum recommended dose in humans. The benefits of therapy must be weighed against the possible hazards to the mother and foetus. It is recommended that nursing should be discontinued while amlodipine is administered.

**Side Effects:**

Amlodipine is well tolerated. Common side effects include headache, edema, fatigue, somnolence, nausea, abdominal pain, flushing, palpitations and dizziness.

Less common side effects are pruritus, rash, dyspnea, asthenia, muscle cramps and dyspepsia. In many instances, the causal relationship is uncertain.

Occurrence of myocardial infarction and chest pain has rarely been reported. However, the relationship cannot be ascertained due to the underlying disease.

**Interactions with Other Medicaments:**

Generally, Amlodipine is well tolerated in co-administration with thiazide diuretics, alpha blockers, beta blockers, angiotensin-converting enzyme inhibitors, long-acting nitrates, sublingual nitroglycerine, non-steroidal anti-inflammatory drugs, antibiotics and hypoglycemic drugs.